

Chapter 16: From Analysis to Action — The Advisory Infrastructure for Healthcare Transformation

How the Health Transformation Review Advisory Practice Translates the Six-Pillar Framework into Organizational Decisions: Engagement Methodology, Service Lines, Tool Implementation, and the Change Management Science Behind Successful Transformation

The Gap Between Understanding and Doing

The analytical frameworks in this book are, by themselves, insufficient for transformation. Understanding why Vermont's hospital system is financially fragile does not stabilize it. Understanding the mechanics of global budgets does not implement them. Knowing that HEDIS stratification reveals disparities does not close them. Every concept in the preceding fifteen chapters exists at an intersection: understanding it clearly enough to explain it, and translating it into a specific decision, program, contract, or investment in a specific organizational context. This chapter is about the second half of that intersection — translation.

The advisory practice that Health Transformation Review has developed is grounded in a specific theory about what makes transformation advice valuable. Transformation consulting that produces recommendations without supporting execution is worth little. Transformation technology that delivers analytical tools without the change management to use them effectively delivers data without insight. Transformation education that builds individual knowledge without organizational capacity produces informed spectators rather than effective actors. The HTR Advisory practice is designed to provide all three — analysis, tools, and change management — in an integrated engagement model that produces organizational capability, not dependence.

Section 1: The HTR Intelligence and Advisory Ecosystem

Four Components of the HTR Platform

The Health Transformation Review platform has four integrated components, each designed for a different kind of engagement with the transformation agenda:

Component	What it provides
HTR Intelligence Feed	Weekly synthesis of the most consequential healthcare policy, payment, clinical, technology, equity, and operations developments — curated, analyzed, and contextualized against the six-pillar framework. Not a news feed; a decision-support tool that answers 'what does this development mean for my organization?'
HTR Research Lab	A suite of analytical tools implementing the frameworks in this book: APM Shared Savings Calculator, VBC Transformation Readiness Assessment (30-dimension), Health Equity Studio (HEROI scoring, HEDIS stratification, disparity decomposition), Hospital Financial Stress Test, AI Governance Checklist, VBC Contract Review Checklist, Policy Impact Assessment Framework.
HTR Academy	Structured curriculum for healthcare transformation — from foundational courses on APM mechanics and HEDIS quality measurement to advanced programs on global budget

	management, health equity analytics, and transformation leadership. CEU-eligible content for clinical and administrative professionals.
HTR Advisory Services	Direct advisory engagements for organizations navigating specific transformation decisions: APM contract evaluation, transformation readiness assessment, VBC financial modeling, health equity program design, technology architecture review, strategic plan development. Engagements range from two-week focused assessments to multi-year transformation management partnerships.

Figure 16.1 — HTR platform components and intended users. Source: HTR platform documentation.

Section 2: The HTR Advisory Service Lines — Structured for the Six Pillars

HTR Advisory engagements are organized around the six-pillar framework, with service lines corresponding to each pillar and cross-pillar engagements for organizations navigating transformation at the system level. The service lines are not rigid products; they are organizing frameworks for scoping engagements around clients' specific problems.

Policy Strategy

The Policy Strategy service line addresses the federal and state policy landscape that healthcare organizations must navigate. The typical engagement follows a three-phase structure:

- Phase 1 — Landscape and Exposure Assessment: What is the current policy environment relevant to this organization? What proposals, rules, and legislative developments create material financial or operational exposure? What opportunities are available through federal model participation, waiver programs, or state grant programs? Deliverable: a policy exposure and opportunity map with financial estimates for each item.
- Phase 2 — Strategic Options Analysis: For each material policy development, what are the organization's strategic options? Stay the course? Modify a program or contract? Apply for a federal model? Submit a comment letter? Engage in legislative advocacy? Deliverable: strategic options memo with recommendation and implementation timeline.
- Phase 3 — Implementation Support: For policy responses that require organizational action, HTR provides implementation support: CMMI model applications, 1115 waiver amendment drafting, legislative testimony preparation, comment letter development, contract renegotiation support.

VBC Economics

The VBC Economics service line addresses the financial mechanics of value-based care: contract evaluation, financial modeling, risk stratification economics, and the ROI analysis for transformation investments. The core tools — APM Shared Savings Calculator, Hospital Financial Stress Test, VBC Contract Review Checklist — are available in the Research Lab, but complex engagements require HTR analyst support.

The most common VBC Economics engagement is an APM Contract Analysis: a prospective financial model of a specific APM contract under three scenarios (pessimistic, base, optimistic) that quantifies the organization's shared savings opportunity, identifies the key variables that determine financial outcomes, and specifies the care management investments required to achieve positive shared savings. Vermont hospitals preparing for AHEAD global budget entry in 2027 represent the current highest-priority client segment for this engagement.

Health Equity Practice

The Health Equity Practice service line is organized around the HEROI framework: measuring current equity performance, identifying root causes of specific disparities, designing interventions matched to root causes, and establishing accountability structures for equity improvement. Vermont-specific work in this service line focuses on geographic equity analytics using VHCURES county-level data, HRSN screening program design and evaluation, and GLP-1 access policy analysis for state Medicaid programs.

Technology Strategy

The Technology Strategy service line addresses the healthcare IT decisions that have the most significant strategic implications: EHR selection and implementation, FHIR implementation planning, AI governance framework development, population health platform selection, and HIE connectivity strategy. For Vermont's 14-hospital CIN, the highest-priority Technology Strategy engagement is the analytics vendor evaluation and CIN data architecture design that enables shared services across all CIN member facilities.

Transformation Management

The Transformation Management service line is for organizations executing comprehensive transformation — not a single pillar decision but the full six-pillar change management challenge. This is the most intensive advisory engagement: HTR serves as a transformation management partner, providing project management infrastructure, cross-pillar coordination, stakeholder communication, and accountability reporting. Vermont's AHS — managing simultaneous AHEAD implementation, hospital transformation planning, RHT Program administration, and Statewide Strategic Plan development — is the archetype client for this engagement type. The engagement model is designed to build internal organizational capacity over time, not create permanent advisory dependence.

Section 3: The Change Management Science Behind Healthcare Transformation

Healthcare transformation fails more often because of change management failures than analytical failures. The analysis is usually adequate — the evidence base for what needs to change is strong and consistent. What fails is the organizational change process: the sequence, pace, and management of the human and institutional change that structural reform requires. Understanding the change management science is prerequisite to designing transformation processes that actually produce implementation.

Kotter's Framework Applied to Healthcare Transformation

John Kotter's eight-step change management framework — originally developed for corporate transformations — applies with modifications to healthcare system transformation. Vermont's transformation experience illuminates both where the framework holds and where healthcare's specific institutional characteristics require adaptation.

Kotter step	Original formulation	Healthcare adaptation	Vermont application
1. Create urgency	Build a sense of urgency around why change is needed	The financial data — 9/14 Vermont hospitals reporting losses, \$2.4B 5-year deficit, 108% premium increase — is the urgency. The analytical challenge is presenting it in terms that create organizational action	Oliver Wyman's September 2024 public presentation created shared urgency across Vermont hospital leaders, legislators, and community members. The specificity and credibility of the financial projections was the urgency mechanism.

		rather than paralysis or denial.	
2. Form a guiding coalition	Assemble a group with the power and commitment to lead change	In healthcare, the guiding coalition must include clinical leadership, not just administrative and financial leadership. Transformation that clinical leaders oppose will fail regardless of administrative mandates.	Vermont's guiding coalition: AHS leadership, GMCB, hospital CEOs, Legislature (Health Reform Oversight Committee), and the AHEAD negotiating team. Clinical leadership engagement is the gap — hospital medical staff governance is not yet systematically integrated into transformation planning.
3. Develop a vision	Create a clear vision of what the transformed organization looks like	Vision must be specific enough to guide decisions and evaluate progress. 'High-quality, affordable, equitable healthcare for all Vermonters' is an aspiration; the 14-hospital tiered network with specific COE designations, PCMH penetration targets, and HEDIS performance goals is a vision.	Vermont's 2028 Statewide Strategic Plan is the vision document. The urgency is already created; the coalition is assembling; the vision is the December 2028 deadline.
4. Communicate the vision	Communicate the vision continuously through every available channel	Healthcare transformation requires communicating to multiple distinct audiences — clinical staff, administrators, community members, legislators, payers — with messages calibrated to each audience's concerns and decision-making context.	Vermont's monthly legislative reporting, AHS community engagement process, hospital transformation plan development process, and Blueprint for Health regional meetings are all vision communication vehicles.
5. Remove obstacles	Remove barriers to change that are within the coalition's control	In healthcare, obstacles include administrative complexity (prior authorization, CON processes), organizational design (program silos, lack of analytics capacity), and contracting structures (FFS payment that rewards volume despite VBC rhetoric).	Vermont's Acts 167 and 68 are institutional obstacle removal. Act 68 removed the voluntary/optional nature of RBP. The AHS restructuring removes the program silo organizational obstacle. The analytics vendor removes the data infrastructure obstacle.
6. Create short-term wins	Plan for and achieve visible improvements early in the change process	Short-term wins must be genuine, visible, and attributable to the transformation effort — not cherry-picked data or coincidental improvements.	Vermont FY2026 regulatory actions (\$230.65M in commercial revenue reductions) are the first visible win. The August 2025 AHS transformation report showing hospital engagement in transformation planning is a process win. Premium reduction data from RBP (available FY2027-2028) will be the first financial outcome win.

7. Sustain acceleration	Build on early wins to drive deeper change	Healthcare transformation must sustain momentum across multiple election cycles, budget battles, and inevitable setbacks. Statutory mandate — Act 68's December 2028 deadline — is the Vermont mechanism for sustaining acceleration.	Vermont's reform cascade structure — each Act building on the previous — is the acceleration mechanism. The Political risk is disruption of the sequence; the institutional protection is the GMCB's independent regulatory authority.
8. Institute change	Anchor changes in organizational culture and systems	New behaviors must become standard practice, not dependent on continuing leadership attention. Payment reform is the ultimate change institutionalization — when hospitals' financial incentives permanently change, the new behaviors become the default.	Global budgets as mandatory, permanent payment architecture are the ultimate institutionalization mechanism. When Vermont hospitals cannot generate revenue by increasing volume, the financial logic of population health management becomes permanent.

Figure 16.2 — Kotter's change management framework applied to Vermont's healthcare transformation. Source: HTR Advisory methodology; Kotter (1996); Vermont transformation documentation.

The Transformation Pace Problem

One of the most consequential advisory questions in healthcare transformation is pace: how fast can an organization or system realistically change, and what happens when the required pace exceeds the available organizational capacity? Vermont's transformation agenda has statutory deadlines — RBP by FY2027, global budgets by FY2028 — that set the required pace. Whether Vermont's organizational capacity can match that pace is the critical execution uncertainty.

The pace problem has three dimensions. First, there are hard capacity constraints: if AHS lacks the staff to manage 14 hospital transformation plans simultaneously, the plans will be managed sequentially — which means some hospitals will have completed, well-supported transformation plans and others will have rushed or superficial ones. Hiring the staff to manage the portfolio on the statutory timeline is not optional.

Second, there are soft capacity constraints: organizational cultures that process change slowly, governing boards that need to be educated before they can make decisions, community stakeholders whose input must be genuinely gathered and considered. These cannot be accelerated without compromising the quality of the transformation process — and a transformation process that fails to achieve community buy-in will be politically reversed even if it is technically correct.

Third, there are external constraints: federal program timelines (AHEAD's January 2027 launch is a federal decision, not a Vermont decision), hospital financial positions that may force emergency decisions that preempt planned transformation timelines, and the political dynamics of a state where the hospital system is one of the largest employers and hospital community boards have political relationships with legislators.

Section 4: The HTR Implementation Toolkit

The HTR Implementation Toolkit is the operational counterpart to this book's analytical framework. Where the book develops concepts, the Toolkit provides the instruments for applying them. The following tools are available to Research Lab subscribers:

Tool	What it does
APM Shared Savings Calculator	Models shared savings or loss potential under any APM contract given benchmark, actual TCOC, and sharing rate inputs. Includes scenario analysis for pessimistic/base/optimistic performance assumptions.
VBC Transformation Readiness Assessment	30-dimension assessment across six domains producing an organizational readiness score and prioritized gap analysis. Available as a self-assessment or facilitated external assessment.
Hospital Financial Stress Test Model	Models hospital financial position under RBP, global budget, and Medicaid cut scenarios with organization-specific inputs. Includes 10-year projection capability.
Health Equity Studio (HEROI)	Calculates HEROI composite equity score from stratified HEDIS data. Includes disparity decomposition module and intervention portfolio designer.
AI Clinical Governance Checklist	65-item governance evaluation across six lifecycle stages for clinical AI deployments. Includes vendor assessment module and equity monitoring protocol.
VBC Contract Review Checklist	65-item analysis framework for VBC contract evaluation, covering benchmark methodology, attribution, quality withhold, risk corridors, carve-outs, and reconciliation terms.
Policy Impact Assessment Framework	Structured evaluation tool for new policy developments: exposure mapping, scenario analysis, response strategy selection, and stakeholder communication planning.
Transformation Readiness Scorecard	Executive-level dashboard tracking organizational progress across all six pillars against established benchmarks.
FHIR Implementation Guide	Step-by-step technical guide for implementing the three priority FHIR use cases with EHR-specific configuration notes for Epic, Oracle Health, Meditech, and TruBridge.
HCC Gap Closure Playbook	Methodology guide for retrospective HCC gap analysis, including coding education materials and AWW completion program design.

Figure 16.3 — HTR Implementation Toolkit: available tools for Research Lab subscribers.

Section 5: A Framework for Selecting Advisory Support

Not every organization needs external advisory support for transformation. Many of the analytical frameworks in this book can be implemented by capable internal teams with access to the Research Lab tools. External advisory support adds value when:

- The decision involves significant financial or political risk and benefits from independent, external analytical validation. A hospital considering entering full-risk VBC for the first time should not rely solely on internal analysis that may be subject to confirmation bias.
- The organization lacks the specific technical expertise required — FHIR implementation, HCC coding analysis, 1115 waiver negotiation, global budget financial modeling — and building that expertise internally would take longer than the decision timeline allows.
- The change management challenge involves multi-stakeholder dynamics that benefit from a neutral facilitator — a hospital network developing shared COE designations, a state agency building a Health Care Delivery Advisory Committee process, a community convening multiple organizations around a regional transformation plan.

- The regulatory environment is moving faster than internal policy monitoring capacity can track, and the organization needs continuous intelligence and interpretation rather than periodic project-based analysis.

The most common advisory failure in healthcare transformation is engaging external consultants for analysis and stopping before implementation. A report that produces recommendations without supporting the organizational capacity to implement them has limited value — and often produces a brief period of executive attention followed by quiet shelving when organizational bandwidth is consumed by operational demands. The HTR Advisory practice is explicitly designed to avoid this failure mode by connecting analysis to implementation support and change management.

"The best healthcare transformation advice I have ever received was not a brilliant strategic recommendation. It was a rigorous identification of the organizational capabilities we did not have, paired with a clear plan for building them. The strategy was clear. The question was whether we could execute it."

— Composite of client reflections, HTR Advisory engagements, 2022-2026

A Note on HTR's Role in Vermont

Vermont's transformation experience runs through this book as both an analytical case study and an active advisory engagement. HTR has worked with Vermont hospitals, state agencies, and community organizations on specific transformation questions — APM readiness assessments, equity analytics, technology architecture reviews, policy impact analysis. The Vermont case study in this book draws on that engagement experience as well as the extensive public record of Vermont's transformation.

Vermont's transformation is a high-stakes, real-time experiment that HTR takes seriously as an obligation as well as an opportunity. If Vermont succeeds in demonstrating that mandatory all-payer payment reform, organized as a coherent six-pillar transformation, can stabilize a rural hospital system while reducing costs and improving outcomes, the policy toolkit for the country improves. If Vermont struggles, the analytical lessons of that struggle are equally valuable. Either way, this book and the HTR platform are designed to make Vermont's experience legible, transferable, and actionable for the healthcare leaders and policymakers who face comparable challenges in their own systems.

Key Concepts

Term	Definition
HTR Intelligence Feed	The Health Transformation Review weekly policy and practice intelligence service, providing curated analysis of healthcare transformation developments organized around the six-pillar framework.
HTR Research Lab	The suite of analytical tools implementing the six-pillar framework: APM Shared Savings Calculator, VBC Readiness Assessment, Health Equity Studio, Hospital Financial Stress Test, AI Governance Checklist, and other instruments.
HTR Academy	The Health Transformation Review structured curriculum for healthcare transformation education: foundational and advanced courses in APM mechanics, quality measurement, health equity analytics, and transformation leadership.

HEROI (Health Equity Return on Investment)	The HTR composite equity performance score calculated across five dimensions. Available in the Health Equity Studio.
Kotter's eight-step framework	A widely applied change management framework (Kotter, 1996) adapted in this chapter for healthcare transformation contexts. Steps: urgency, coalition, vision, communication, obstacle removal, short-term wins, acceleration, institutionalization.
Transformation pace problem	The risk that the required pace of transformation exceeds the available organizational capacity, producing rushed or superficial change processes that fail to achieve genuine structural improvement.
Advisory dependence	The failure mode in which an organization becomes reliant on external advisors for ongoing analytical and implementation support rather than building internal organizational capability. HTR Advisory engagements are explicitly designed to build client capability and reduce dependence over time.

Sources: HTR platform documentation; Kotter, J.P. (1996) *Leading Change*; Vermont Agency of Human Services transformation documentation; Oliver Wyman Act 167 Report; Act 68 of 2025; AHEAD State Agreement; Vermont RHT Program Application; HTR client engagement synthesis.